

Interventions to enhance symptom management and quality of care in hospitalized patients at the end-of-life: a scoping review

Andrea M. Yeung, BA¹, Max T. Hukill, AB¹, Scott Y. Johnson, EdD, MLIS, AHIP¹, Susan E. Wang, MD², Andre Cipta, MD^{1,2}

¹Kaiser Permanente Bernard J. Tyson School of Medicine, Pasadena, CA
²Department of Geriatrics and Palliative Medicine, Kaiser Permanente Southern California, West Los Angeles, CA

Introduction

The assessment of symptom management comprises a core element of high-quality end-of-life care in older adults, which drives innovation in this area. We investigate the existing literature for interventions and guidelines to enhance symptom management and care for hospitalized patients at end-of-life.

Objective

To investigate the existing literature for practices, innovative interventions, and guidelines to enhance symptom management and quality of care for hospitalized patients at end-of-life.

Methods

- A multifactorial search query was conducted in PubMed, CINAHL, and Cochrane Database of Systematic Reviews with a carefully crafted search string
- Inclusion criteria:
 - Inpatient or hospital setting
 - Patients in study prioritized comfort-focused care
 - Population is at the end of life
 - Articles from 2000-2024
- Articles were screened for relevance to symptom management or quality of care interventions at end-of-life in hospitalized patients
- Manual abstract reviews and citation tracking were conducted to expand the search and include empirical studies, systematic reviews, and clinical guidelines

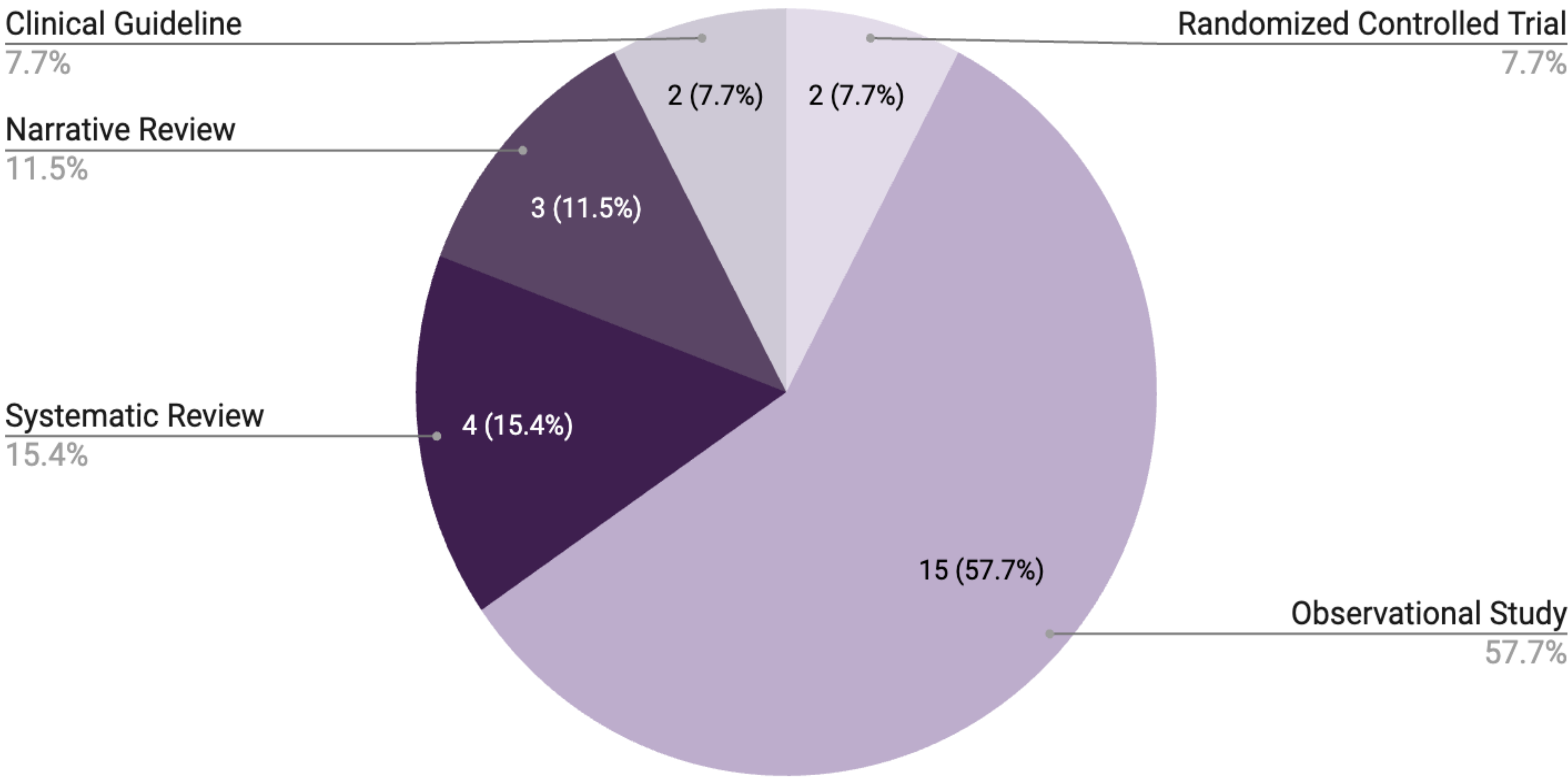


Figure 1. Types of studies identified in scoping review

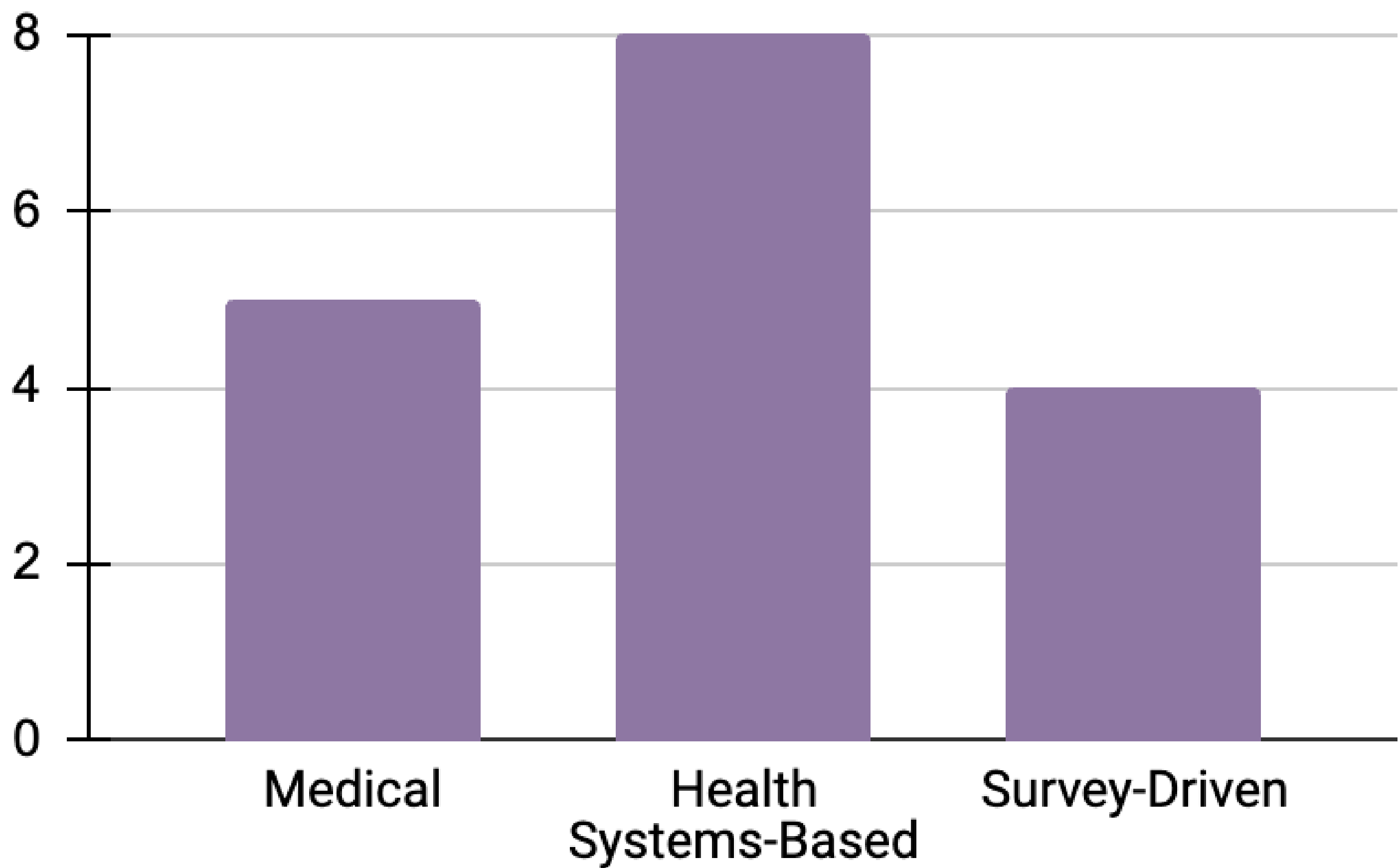


Figure 2. Types of interventions identified in reviewed articles

Results

Twenty-nine articles met the inclusion criteria, including randomized controlled trials (n=2) and observational studies (n=15) investigating specific interventions, which were categorized as medical (n=5), health systems-based (n=8), and survey-driven (n=4). Recurring themes included patient comfort, communication, supportive care needs, and symptom management. The remaining articles included systematic reviews (n=4), narrative reviews (n=3), and clinical guidelines (n=2). The majority of studies were conducted in the United States (n=14), with fewer studies from Canada, Europe, Australia, East Asia, and South America. Four articles specifically described and evaluated metrics used to assess quality of life and symptom management at the end-of-life. Notably, only one study incorporated medical trainees in intervention implementation, highlighting an opportunity for future innovation.

Conclusion & Implications

This review highlights a diverse range of interventions designed to enhance symptom management and quality of care for hospitalized patients at the end-of-life. Although progress has been made, significant gaps remain in standardizing assessment of symptoms and distress at end-of-life. Future efforts should focus on identifying health systems-based and survey-driven interventions to drive quality of end-of-life care, with an emphasis on engaging trainees to drive quality improvement.